Confidentiality, Access and Medico-Legal Requests

Who may see the record, who may be given it, how to answer the police and the court, and how to document an MLC.

Module	11 of 12
Study time	105 minutes
Audience	Medical records staff and officers, clinicians, nursing, administration, legal and compliance teams
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	DPDP Act 2023; HIV/AIDS Act 2017; Mental Healthcare Act 2017; Code of Criminal Procedure / BNSS; NABH 6th Edition PRE and IMS chapters

A hospital holds some of the most sensitive information that exists about a person. The duty to keep it confidential is professional, contractual and now statutory, and it has a small number of precise exceptions. Most breaches in Indian hospitals are not malicious; they are procedural — a file handed over because a man in uniform asked, a report discussed at a nursing station, a summary emailed to an employer. This module covers who may be given what, on what authority, and how to document the decision.

LEARNING OUTCOMES — by the end of this module you will be able to:

- State the legal and professional basis of the duty of confidentiality in India
- Determine who may be given a record, and on what authority, in each common situation
- Operate a release-of-information workflow with verification, scope limitation and logging
- Respond correctly to a police requisition and to a court summons
- Apply the additional confidentiality rules for HIV, mental health, MTP and sexual assault records
- Identify a medico-legal case, make the entries required, and preserve material evidence
- Recognise and report a data breach involving health information

11.1 The basis of the duty

Four sources converge. Professionally, the IMC Regulations require a doctor to protect confidences learned in the course of care and permit disclosure only in defined circumstances. Contractually, the hospital undertakes to keep information confidential and binds its staff to the same. Statutorily, the DPDP Act 2023 treats health data as personal data and imposes obligations of purpose limitation, security and breach reporting. And several specific statutes impose stricter duties for particular categories of information.

The duty binds everyone, not only clinicians: nursing, medical records, billing, housekeeping, security, IT and contracted staff. Every one of them should have signed a confidentiality undertaking, and every one of them should know that discussing a patient outside the clinical need to know is a disciplinary matter.

CAUTION
The commonest breach in a hospital is a conversation. Lifts, corridors, canteens and nursing stations within earshot of waiting relatives account for more disclosure than any file ever does. The second commonest is a screen left logged in and facing a public area.

11.2 Who may be given the record

Who may be given the record, and on what basis

Requester	Normally entitled?	What the MRD must see or do
The patient	Yes	Identity proof; written request; copy supplied within 72 hours per IMC Reg 1.3
Spouse, parent, adult child	Not automatically	Written authorisation from the patient, unless the patient lacks capacity or has died and the requester is the legal heir
Legal heir of a deceased patient	Yes, with proof	Death certificate, proof of relationship, identity; release logged
Treating clinician of another hospital	Yes, for care	Patient's consent where practicable; a referral letter and a documented clinical purpose
Insurer or TPA	Only with authorisation	The authorisation signed by the patient in the policy or claim form; release limited to the episode claimed
Police	Only as the law provides	Written requisition under section 91 CrPC / section 94 BNSS naming the record; MLC records released; non-MLC needs consent or an order
Court	Yes, on summons	Original produced by a named officer; certified copy retained; never hand over without a receipt
Employer, media, relatives generally	No	Refuse politely, in writing, citing confidentiality; refer to the patient

Every release is logged: who asked, under what authority, what was given, by whom, when, and to whom it was handed.

Figure 11.1 — Who may be given the record, and on what basis.

The default is that nobody other than the patient receives the record, and every departure from that default requires either the patient’s authorisation or a specific legal power. The role of the medical records officer is to establish which applies before anything is released.

Family members

An adult patient’s spouse, parent or adult child has no automatic entitlement. This surprises people and is frequently resisted, but it follows directly from the fact that the information belongs to the patient. Where the patient lacks capacity, release to the person making decisions on their behalf is appropriate and the basis should be recorded. Where the patient has died, the legal heir may be given the record on proof of death and of relationship.

Insurers and third-party administrators

Release is made on the authorisation the patient signed in the policy or the claim form, and it is limited to the episode being claimed. An insurer asking for the patient’s entire history going back years, in order to look for a pre-existing condition, is asking for more than the authorisation covers; the hospital should release what relates to the claim and direct any wider request to the patient.

Employers, schools, media and others

No entitlement. The request is declined politely, in writing, citing confidentiality, and the requester is directed to the patient. This includes requests that come with apparent authority — an employer’s HR department, a journalist with a public-interest argument, a politician’s office. None of these creates a legal power to receive health information.

11.3 The release-of-information workflow

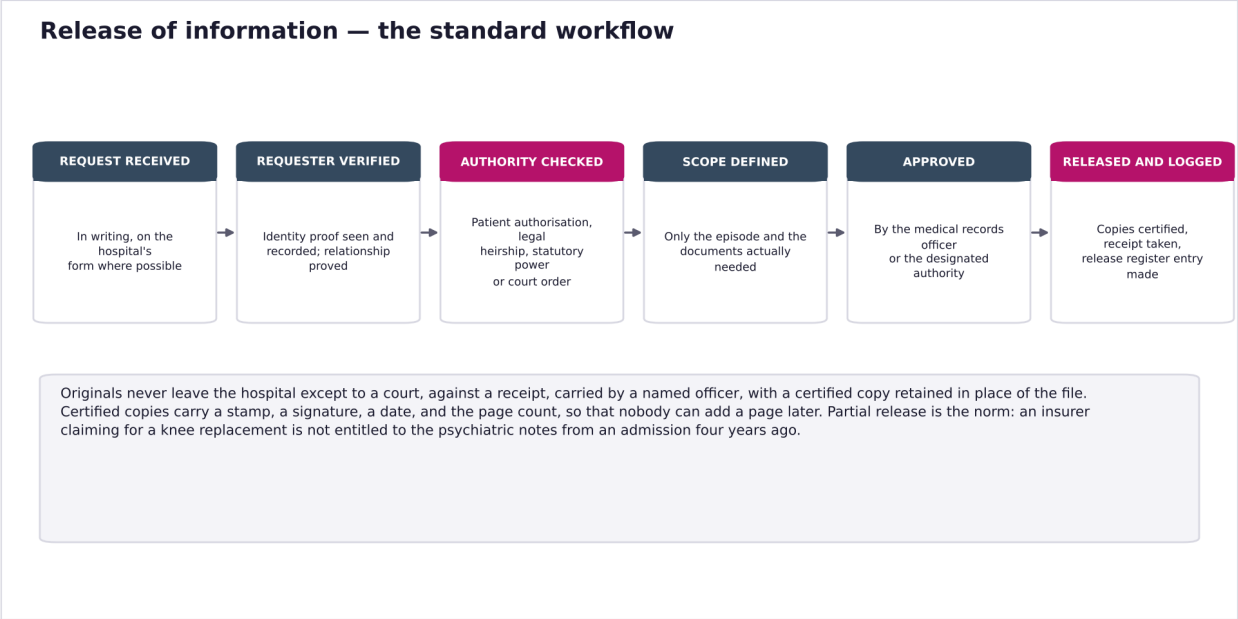


Figure 11.2 — The standard release workflow.

1. **Request in writing.** On the hospital's form where possible, stating what is wanted and why.
2. **Verify the requester.** Photo identity seen and recorded; relationship proved by document where it is relied upon.
3. **Check the authority.** Patient authorisation, legal heirship, a statutory power, or a court order. Record which one applies.
4. **Define the scope.** Only the episode and the documents actually needed. Partial release is the norm, not the exception.
5. **Approve.** By the medical records officer or the designated authority, not by whoever is at the counter.
6. **Release and log.** Copies certified with a stamp, signature, date and page count; a receipt taken; the release register entry made.

Certification of copies matters more than it appears to. A stamp, a signature, a date and a stated page count mean that nobody can add a page to the set afterwards and attribute it to the hospital. Originals do not leave the hospital except to a court, against a receipt, carried by a named officer, with a certified copy retained in their place.

11.4 Police requests

A police officer may require production of a document by a written requisition under the applicable criminal procedure provision — section 91 of the Code of Criminal Procedure, now section 94 of the Bharatiya Nagarik Suraksha Sanhita. The requisition must be in writing and must identify the record sought. A verbal demand, however insistent, is not a legal requirement to produce.

Situation	What to do
Medico-legal case; written requisition produced	Provide the record or certified copies as required; log the release with the officer's name, rank, station, requisition number and date
Non-medico-legal case; written requisition produced	Provide what the requisition specifically requires; where it is broad, seek guidance from the hospital's legal adviser and record having done so
Verbal demand, no written requisition	Explain politely that a written requisition is required; offer to provide it immediately on receipt; record the interaction
Request for information about a patient who has not been declared an MLC	Do not disclose without the patient's consent or a written requisition; declare the MLC if the case meets the criteria

Situation	What to do
Request to delay or withhold treatment pending formalities	Treat the patient. Emergency treatment is never delayed for police formalities; record the request and the fact that treatment proceeded
Officer asks to speak to the patient	A clinical decision about whether the patient is fit to be interviewed; record the decision and who made it

11.5 Court summons and giving evidence

A summons to produce records is complied with. The record is produced by a named officer, against a receipt, and a certified copy is retained so that the hospital continues to hold a complete set. Where the summons requires a witness, the person who attends should have personal knowledge or be the custodian able to speak to the authenticity of the record.

- Check what the summons actually requires: production of documents, attendance, or both
- Identify the correct custodian; do not send a junior clerk with a file they cannot speak to
- Take the original, a certified copy set, and the index
- Obtain a receipt listing what was handed over, page by page where feasible
- Record the court, the case number, the date, what was produced and to whom
- For electronic records, take the certificate required for admissibility, signed by the person authorised to give it
- Never annotate, tidy or complete a record because it is going to court; a correction made after a summons is the worst possible fact in any case

> **CAUTION**
> Altering a record after notice of a claim or a summons converts a civil dispute into an allegation of fabrication and exposes individuals personally. If a record is incomplete, it goes as it is, and the gap is explained in evidence.

11.6 Categories with stricter rules

Category	Additional rule	Practical handling
HIV status	The HIV and AIDS Act 2017 restricts disclosure to the person tested, with narrow exceptions; consent and counselling are required before testing	Results stored so that they are not visible to anyone handling the general record; disclosure only on the statutory basis
Mental health	The Mental Healthcare Act 2017 gives a right to confidentiality of mental health information with defined exceptions	Restricted handling; the nominated representative's role recorded; disclosure justified against the statutory exceptions
Termination of pregnancy	The MTP framework protects the identity of the woman	The register and records kept separately and securely; identity not disclosed except as the law permits
Sexual assault examination	Records and samples form evidence and attract strict handling	Chain of custody documented; release only to the investigating authority or the court; the survivor's identity protected
Genetic and prenatal diagnostic records	The PCPNDT Act imposes specific record-keeping and prohibits disclosure of foetal sex	Statutory forms maintained; records retained for the period the Act requires and produced only to the appropriate authority
Records of minors in protection matters	Child protection law restricts disclosure of identity	Handled through the designated officer; identity protected in all correspondence

11.7 Medico-legal cases

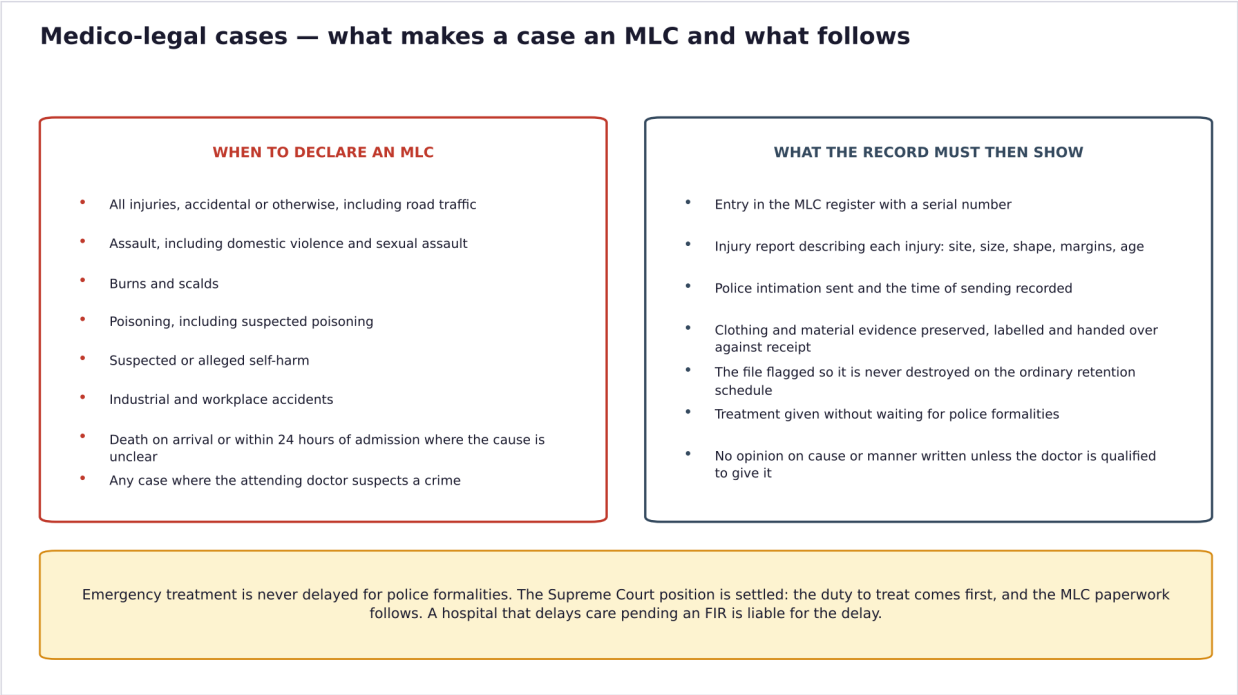


Figure 11.3 — What makes a case medico-legal, and what the record must then show.

An MLC declaration is a documentation act with consequences that run for years: the file is flagged, it is excluded from routine destruction, and it will probably be read by a court. The declaration should be made by rule, using the hospital's criteria, rather than by individual judgment, because the cost of under-declaring is far higher than the cost of over-declaring.

The injury report

Where injuries are present, each must be described separately and objectively: site measured from fixed anatomical landmarks, size in centimetres, shape, margins, colour, depth where ascertainable, and the apparent age of the injury. Clinical opinion should be confined to what the examiner is qualified to give. A doctor who is not a forensic specialist should describe the injury and should not opine on the weapon used or the manner of infliction.

- Record the time of examination, not only the date
- Record the history as given, attributed to the person who gave it, in quotation marks where it matters
- Record an explicit statement where an expected finding is absent
- Preserve clothing and material evidence, labelled, sealed and handed over against a receipt
- Record the time the police were informed and by whom
- Make no alteration to the injury report once signed; any correction follows the standard method and is initialled, dated and timed

> **KEY POINT**
> Emergency treatment is never delayed for police formalities. The duty to treat comes first and the medico-legal paperwork follows. A hospital that delays care pending an FIR is liable for the delay, and the record should show that treatment began immediately.

11.8 Data breaches

A breach is any unauthorised access, disclosure, alteration or loss of personal data. In a hospital that includes a lost file, a misdirected email or report, an unauthorised access to an electronic record, a stolen laptop, a

photograph shared outside the hospital, and a ransomware incident.

1. Contain: stop the exposure, recover what can be recovered, revoke access.
2. Assess: what data, how many individuals, what sensitivity, what consequences.
3. Report internally at once, through the incident system and to the designated officer.
4. Notify as the law requires, including the Data Protection Board and affected individuals where applicable.
5. Record the whole sequence, including the timeline of discovery, which is what any later examination will focus on.
6. Investigate the cause and act on it; a breach caused by shared logins is fixed by ending shared logins, not by counselling the individual.

11.9 Privacy in the physical environment

Most confidentiality breaches happen in the open, not in the record room. They are also the easiest to fix, and they are what patients notice.

Where it happens	What to change
Nursing stations within earshot of waiting relatives	Move clinical discussion away from the counter; position the station so conversations are not overheard
Handover at the bedside in an open ward	Use a structured handover away from the bedside for sensitive content, with bedside handover for the patient-facing part
Whiteboards showing diagnoses	Display only what is operationally necessary: bed, name where policy permits, and care flags that are not diagnostic
Screens facing public areas	Reposition, apply privacy filters, and enforce locking when a workstation is left
Files open on trolleys	Files closed when not in use; the jacket carries no diagnosis on the outside
Consultations in shared spaces	Curtains at minimum, and a private room for sensitive discussions, particularly breaking bad news
Discharge counselling at a busy counter	A designated space where medication and follow-up can be explained without an audience

11.10 Telephone and electronic enquiries

A caller cannot be identified over the telephone, which makes telephone disclosure the easiest breach in a hospital to commit and the hardest to defend.

- Do not disclose clinical information to an unverified caller, however plausible they sound
- Where the hospital permits telephone updates, operate a pre-agreed arrangement: a nominated family member and an agreed verification method recorded at admission
- Offer to take a number and call back on a number already in the record, which is the simplest verification available
- Do not confirm or deny that a named person is an inpatient where the patient has asked for that, and record such requests at admission
- Treat email and messaging as equivalent to any other disclosure: verify the recipient, limit the content, and use the hospital's systems rather than personal accounts
- Record every telephone disclosure: who called, how they were verified, what was disclosed, and by whom

11.11 Staff, social media and curiosity

Two staff behaviours generate most internal breaches. The first is curiosity — looking up the record of a colleague, a relative, a neighbour or a well-known person. The second is discussing a case on social media or a messaging

group, often with identifying details the author did not think were identifying.

- Access without a clinical need to know is unauthorised access, whatever the motive, and the audit trail will show it
- Looking up your own record through the clinical system is also unauthorised; request it through the proper route
- Nothing about a patient goes on social media, including anonymised cases where the combination of details identifies the person locally
- Departmental messaging groups are not a clinical record and should not carry patient identifiers or images
- Teaching material must be de-identified, and the consent for teaching use recorded
- A staff member who finds themselves caring for someone they know should declare it and the record should show the arrangement made

> **CAUTION**
> Run a periodic audit-trail review for access to the records of staff members, of patients with the same surname as staff, and of any admission that attracted attention. Hospitals that do this find breaches; hospitals that do not have them anyway.

11.12 High-profile and sensitive admissions

A well-known patient, or any admission that attracts public interest, needs a plan made in advance rather than improvised at the front desk.

1. Restrict electronic access to the named care team, with a break-glass route that is logged and reviewed.
2. Use a documented pseudonymisation approach only where the system supports it safely; a renamed record that the care team cannot find is a safety risk.
3. Brief all staff that no information whatsoever is confirmed to anyone, including the existence of the admission.
4. Route all external enquiries to a single named spokesperson.
5. Monitor the audit trail in real time for unauthorised access.
6. Record the arrangements made, so the hospital can show what controls were in place.

11.13 Research access to records

Records are a research resource and access to them is governed. The researcher's convenience is not a basis for access, and a department's own audit is not automatically research.

Purpose	Basis for access	Conditions
Clinical audit of the hospital's own service	Service evaluation under hospital governance	Minimum data, no identifiers leaving the hospital, results reported in aggregate
Research involving record review	Institutional ethics committee approval	Approved protocol, defined data set, waiver of consent only where the committee grants it, defined storage and destruction
Research involving contacting patients	Ethics approval and patient consent	Approach route approved by the committee; the care team, not the researcher, makes first contact where required
Teaching and case presentation	De-identification	No identifiers; consent where the case is identifiable; images consented for teaching
External collaboration or data sharing	Ethics approval plus a data sharing agreement	Data minimised and de-identified; purpose, duration and destruction defined; the hospital remains accountable

11.14 Responding to a complaint about confidentiality

1. Treat it as an incident from the first report, and start a timeline immediately.
2. Establish the facts from the audit trail and the release register rather than from recollection.
3. Contain any continuing exposure before investigating the cause.
4. Tell the patient what happened, what information was involved, and what is being done, promptly and in writing.
5. Report externally where the law requires, and record the reporting decision and its reasoning either way.
6. Fix the cause, which is usually a process or an access-control defect rather than an individual.
7. Record the whole sequence; how quickly a hospital noticed and responded is what any later examination will focus on.

11.15 Consent-based sharing and the digital health framework

The Ayushman Bharat Digital Mission builds a consent-based mechanism for sharing health records between institutions, linked to a patient's ABHA number. The hospital's obligations under it run alongside, not instead of, its existing confidentiality duties.

- Linking a record to an ABHA number requires the patient's consent, recorded
- Each sharing request is consented separately, for a defined purpose, scope and period
- The hospital must be able to show what was shared, with whom, under which consent artefact, and when
- Withdrawal of consent must be actionable, and its effect on data already shared explained to the patient
- The DPDP Act obligations — notice, purpose limitation, accuracy, security safeguards and breach reporting — apply to all of this
- Staff must understand that a system-mediated share is still a disclosure and is logged as one

The practical consequence for documentation practice is that records will increasingly be read outside the institution that wrote them, by clinicians with no access to the context. That raises the standard for clarity: abbreviations that are local, diagnoses that are vague and entries that depend on knowing the ward will not survive the journey.

11.16 Requests from other hospitals

A request from another treating institution is a continuity-of-care request and should be handled quickly, but it is still a disclosure and still requires a basis.

Situation	How to handle it
Patient is currently being treated elsewhere and the clinician requests records	Obtain the patient's consent where practicable; where the patient cannot consent and the information is needed for their immediate care, release the clinically necessary minimum and record the basis
Emergency request for a patient who is unconscious at another hospital	Release what is clinically necessary — allergies, current medication, implants, recent procedures — and record the request, the requester, the verification and what was sent
Routine request with no urgency	Written request, patient consent, verification of the requesting institution, and release through the normal workflow
Request arriving by email or messaging from an unverified address	Verify through the institution's published contact details before sending anything; never reply with clinical content to an unverified address
Request for a patient who was never treated here	Do not confirm or deny; ask the requester to check the identifiers with the patient

KEY POINT
Build a fast lane for genuine clinical continuity requests, with verification, so that staff are not forced to choose between a patient's immediate safety and the hospital's confidentiality rules. Where no fast lane exists, staff improvise, and the improvisation is the breach.

11.17 Putting this module to work

1. Find out who in your hospital is authorised to approve a release of information, and whether the counter staff know.
2. Ask to see the release register. Does each entry record the authority relied upon?
3. Check whether your hospital has a standing MLC criteria list, or whether the declaration is left to individual judgment.
4. Check whether HIV results and mental health records in your system are visible to everyone with general record access.
5. Find out what the hospital would do if a police officer appeared at the counter tonight with a verbal demand for a file.

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A man presents at the medical records counter saying his wife was treated last month and he needs her records for an insurance claim. She is alive and well. What do you do?**

Answer: Her consent is required. Explain that the information belongs to her, ask for her written authorisation together with her identity proof and his, and confirm what she is authorising release of. If the claim is under her own policy, the authorisation she signed in the claim form may cover release to the insurer directly; it does not authorise release to her husband. Record the request, the authority relied upon, and what was released.

Q2. **A police officer arrives without a written requisition and demands the file of an accident victim, saying it is urgent. What is the correct response?**

Answer: Treat the case as medico-legal if it meets the criteria, and make the MLC entries and police intimation regardless. For the record itself, explain politely that a written requisition under the applicable criminal procedure provision is required and that the hospital will provide the record immediately on receipt. Record the interaction, including the officer's name and the time. Nothing about this delays the patient's treatment, which continues throughout.

Q3. **An insurer asks for the patient's complete records for the last ten years to assess a claim for a knee replacement. How much should be released?**

Answer: Only what relates to the episode claimed, because that is the scope of the authorisation. A request for ten years of records is a search for a pre-existing condition and goes beyond what the patient authorised. Release the episode, note the limitation in the release register, and direct the wider request to the patient, who can decide whether to authorise more. Releasing unrelated history, particularly psychiatric or reproductive health records, is a breach.

Q4. **A hospital receives a summons to produce a file. The treating doctor notices that one progress note is unsigned and signs it before the file is sent. What is the effect?**

Answer: It converts a minor deficiency into an allegation of fabrication. The signature is demonstrably not contemporaneous, and once one entry is shown to have been added after notice of proceedings, every entry in the file is treated with suspicion. The file should have gone as it was, with the gap explained in evidence as an omission. The doctor has now exposed himself personally in addition to damaging the hospital's case.

Q5. **A nurse looks up the record of a colleague who was admitted overnight, out of concern. She tells no one what she read. Is this a breach?**

Answer: Yes. Access without a clinical need to know is unauthorised access, regardless of motive and regardless of whether anything was disclosed onward. The audit trail will show it, and in most hospitals it is a disciplinary matter. It is also a reportable incident under the hospital's data protection arrangements. The fact that the intention was kind does not change the position; the same reasoning would permit anyone to read anything.

Q6. **A doctor examining an assault victim writes in the injury report that the wound was 'caused by a sharp weapon, probably a knife, from behind'. What is wrong with this?**

Answer: It goes beyond what the examining doctor is qualified to state and beyond what the examination can establish. The report should describe each injury objectively — site measured from fixed landmarks, size, shape, margins, depth, apparent age — and leave inferences about weapon and manner to a forensic expert and ultimately the court. An unsustainable opinion in an injury report is challenged in cross-examination and weakens the credibility of the whole report, including the reliable parts.

ASSESSMENT BANK
A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.